DEMENTIA

Dementia and home safety: routine, wandering, and keeping changes familiar

By Russ L'HommeDieu, DPT, ATP · Doctor of Physical Therapy and RESNA Assistive Technology Professional · November 2, 2026



Dementia and mild cognitive impairment double the risk of falling and of getting hurt in a fall. For a parent with memory changes, the safest home is predictable: the same routes, the same routines, and changes that feel familiar rather than strange. Caregivers are a key part of the plan, not an afterthought.

Why falls are more likely with dementia

Along with memory changes, your parent may have:

- **Poor safety awareness**, such as forgetting the walker or standing up too fast.
- **Impulsivity**, acting before thinking a move through.
- **Trouble planning and sequencing** a task (executive dysfunction), like getting from the bed to the toilet.
- **Wandering**, including at night or out an exterior door.
- **Delirium**, a sudden increase in confusion. That is a reason to call the doctor.

Routine is a safety tool

Routines, repetition, and cueing are core strategies for people with cognitive impairment. Movement becomes safer when it happens the same way every time.

- **Same path, same time.** Walk the same route through the house each day.
- **Same words.** Use the same short cues for every transfer, such as "scoot forward, feet back, stand." Everyone who helps should use the same wording.
- **Build movement into chores.** Folding laundry, setting the table, and putting away groceries keep your parent moving in a familiar way.
- **Use rhythm.** Clapping, counting, or a favorite song can help time a movement.
- **Practice sit-to-stand.** Standing up from a chair with cues, three times a day, builds leg strength that matters for every transfer.

A helpful phrase from the PESI Fall Prevention Strategy Guide: "We're doing these moves the same way each day to help your body remember."

Keep changes familiar

Because routine matters so much, how you change the home matters as much as what you change. Fit new items into what your parent already does.

- **Put new things where old habits already go.** A grab bar belongs at the spot where your parent already reaches when sitting on the toilet.

- **Keep the layout steady.** Clear clutter and open paths, but avoid rearranging furniture again and again.
- **Use signs and pictures.** Clear, bold signs for the bathroom, kitchen, and exits, and labels with both words and pictures on drawers, doors, and closets.
- **Add contrast.** Contrasting colors on furniture edges, stairs, and the toilet seat make them easier to see.
- **Calm the space.** Remove clutter and busy, overstimulating decor.
- **Help with time.** Put clocks and calendars in several rooms, and use a simple visual schedule board.

Light the night

Night trips to the bathroom are among the riskiest moments of the day. Add motion sensor night lights in the bedroom, hallway, and bathroom, and light the whole route. Keep glasses, water, and a flashlight by the bed.

A plan for wandering

If your parent wanders, the home needs a plan, not just a lock:

- **Limit access** to stairs and exterior doors where wandering is a risk.
- **Consider bed or chair alarms** if your parent may leave the home unsafely.
- **Watch the lanai slider.** In many Florida homes it is both an exit and a trip point at the track.
- **Respect dignity.** Any monitoring should be done with consent where possible, never with cameras in the bathroom.

Exercise still works

The World Guidelines for Falls Prevention strongly recommend exercise for people with mild cognitive impairment and mild to moderate dementia. Balance training such as tai chi and programs that combine strength and balance are good examples. The guidelines also say plans work better when caregivers help find home hazards, shape daily exercise and meal routines, keep a record of any falls, and decide what matters most to the person, including how much risk they are willing to accept.

Medicines and red flags

Some medicines can worsen confusion in people with dementia. Bring a full list of medicines and supplements to the doctor or pharmacist for review, and never stop or change one on your own.

Stop and reassess if you see:

- More confusion or agitation during movement
- Wandering, impulsivity, or loss of safety awareness
- New trouble with familiar transfers
- A medicine or behavior change that affects daily routines
- A caregiver who feels overwhelmed

Support for the caregiver

Memory care teams, neurology or primary care, an occupational therapist, and caregiver support or respite programs can all share the load. In Tampa Bay, the Senior Connection Center and the Area Agency on Aging of Pasco-Pinellas connect families with local aging services. The Florida Elder Helpline is 1-800-963-5337.

How Home Safety Solutions can help

A home visit for a parent with dementia looks at daily routines first, then the routes, lighting, bathroom, and exits that support them. Recommendations are ranked so you can start with free, familiar changes. Learn more about what happens during a home safety visit. Call (813) 365-9171 or email info@homesafety.solutions.

Related reading: Can my parent age in place in their current home? · The Quiet Negotiation: Safety, Independence, and Compassion · CDC caregiver brochure (PDF) · Is it normal to fall as you get older?

This article is general education, not medical advice. Talk with your doctor, pharmacist, or therapist about your own situation.

Want a plan for your own home?

Every service starts with an Initial Consultation, a 30-minute phone call with Dr. L'HommeDieu.
Book at homesafety.solutions